



The Central African Republic has been burdened with wars and dictatorships since the 1960's, and despite being endowed with internationally coveted resources such as expansive diamond mines, 73% of its population live on less than US \$1 a day. During the past 15 years, life expectancy has declined from 45 to 35 years of age due to poor health care and the HIV/AIDS epidemic which plagues 35% of the CAR's population. Moreover, the lack of essential drugs and vaccinations has contributed to the recent outbreaks of other diseases such as malaria, tuberculosis, polio and water-borne viruses. The World Health Organization's resident representative in the Central African Republic stated in a recent interview that the CAR's health system is "very precarious," adding that most health workers in rural areas have abandoned their posts because of the region's insecurity and unpaid wages.

Like Zimbabwe and other African nations, the "Brain Drain" migration of health care workers and other trained professionals to the more developed world is significantly contributing to the Central African Republic's current health situation. Presently, the UN is appealing for \$45 million for humanitarian assistance for the CAR, however, the nation anticipates receiving less than a third of that. The CAR recognizes that although there is a need for vaccinations and other salutary goods, the health care system will never be able to reach the thousands of people in need without the physical manpower provided by doctors, nurses and other healthcare workers.

At the center of the nation's policy is the belief that without an international effort, there is no way of solving an international problem. The Central African Republic proposes a resolution which mandates that the subsidy paid by governments to train a doctor or nurse should be calculated, and repaid in an exact amount to the government if that person chooses to migrate. The cost would be paid by the country to which the person is migrating, as it is benefiting from the transfer of "human capital" trained at great cost by the country of origin. However, the CAR also recognizes that income alone will not be sufficient recompense for the lost immigrants and we therefore propose that in developing countries – with a population of migrating workers over a certain percentage – that trained individuals would have to serve a short number of years in their country of origin before being allowed to migrate to practice in a different country.

The CAR also recommends that the UN channels funds through organizations like UNESCO, OECD and the World Bank to help improve the wages and living conditions of healthcare workers in third world countries, (like the CAR), who have no other means of financially motivating their citizens to remain and practice in their own nation. The CAR also plans on advocating for a higher percentage of the national GDP to be devoted to research, development, and the promotion of the sciences. Additionally, as an African nation encourages the whole continent to forge a partnership with the diaspora community of Africans worldwide. An effective partnership implies building capacity and exploiting existing capacity within the existing network of highly skilled and expertise African expatriates. Finally, the Central African Republic proposes a resolution that includes the creation of a database on brain drain and capacity building, thus creating knowledge blocks to complement national efforts in addressing deficiencies in education and training capacities as well as trying to maintain and effectively utilize them.